ROYAL CANADIAN NAVY MARINE ROYALE CANADIENNE

CASUATLY CLEARING TEAM TRAINING

Briefer

CANADIAN ARMED FORCES

FORCES ARMÉES CANADIENNES

Briefing to HMCS *ship*

Date: DD-MMM-YY

WELCOME TO DCTF GALIANO (CCOTT)

DCTF

- **One of the largest integrated fire fighting trainer in the world:**
  - 15 burns room
  - 2 flood tanks
  - Fire and Flood Training
  - Casualty Clearing Training
  - NBCD Training
  - DCOTT

Fire Training Area Safety Rules

- The following pipe will be heard during the day:
- “Caution is to be exercised in the training area live fire training in progress”

Room Stops

“E” Stops

Flashover

Safety Doors

Training Boundary - Do not cross

Fire Alarm

- Lead Instructor will direct all trainees to exit building
- Class Leader (PA) will muster class off roadway in front of Building 94 (report to Lead Instructor)

Emergency Evacuation

- Pipe will be heard
- “EMERGENCY EVACUATION, EMERGENCY EVACUATION, (REASON STATED) ALL PERSONNEL MUSTER OFF ROADWAY OUTSIDE BUILDING 94” PIPE WILL BE REPEATED
- Follow direction of Safety/ Instructor

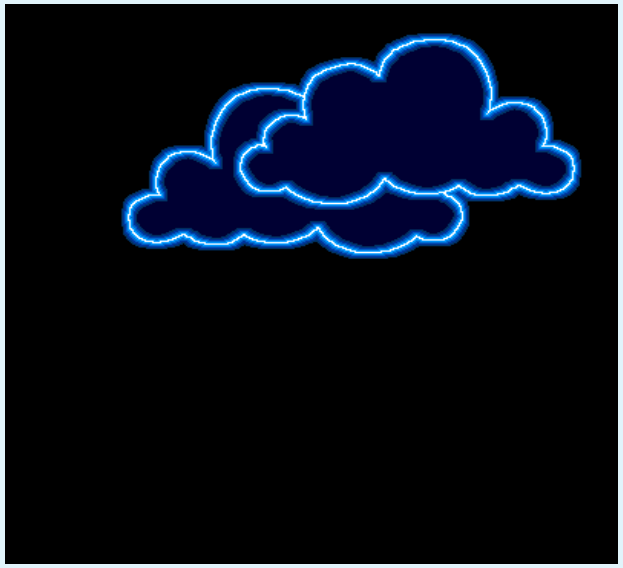
Earthquake

- STOP Training
- Wait 30 seconds after shaking stops
- Follow direction of Safety/ Instructor
- Evacuate building and muster outside of Building 94

Power Failure

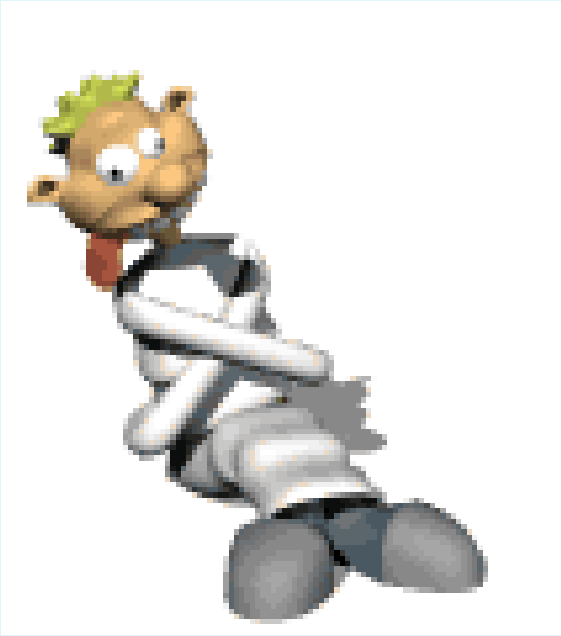
- All Training to

- Follow directions of Instructor/ Safety personnel



SAFETY

- It is recommended Contact lenses be removed for safety reasons.
- Personnel with medical conditions ie; asthma,bronchitis, heart condition, or skin condition are to inform their Physician Assistant.



SAFETY

- Any accidents or injuries must be immediately reported to instructor in charge of the class.

CLASS LEADER - PHYSICIAN ASSISTANT

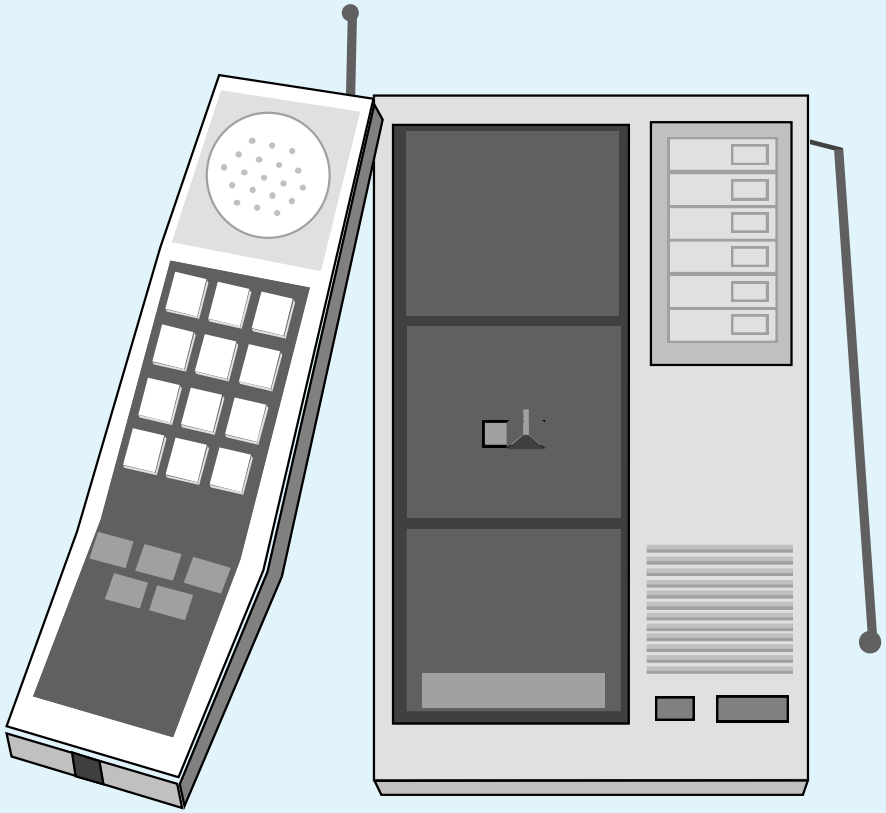
- Responsible to ensure that all personnel are present and on time according to the clock at the back of the classroom.
- All chairs are placed on the desks at the end of the day.
- Complete the course critique ensuring that the classes comments are recorded.
- You are in charge.

SCHOOL HOURS

- MON - FRI : 0800-1530. LUNCH: 1130 - 1230.
- REGULATING OFFICE PHONE #363-2837

MISCELLANEOUS

- A telephone for students use is available between the locker rooms in the DCTF.



LOCKER ROOMS

- Male and Female locker room and showers are on the ground floor to the left of the main entrance in the DCTF trainer

SMOKING AREA

- The designated smoking area is marked area, concrete half-circle between the buildings in the outdoor break area.

Canteen Items Are Available

Information Management & Guidance

- This presentation is UNCLASSIFIED
- This document is the property of the RCN and shall not be altered, copied and / or distributed without the document owner's permission
- The document owner is Sea Training Group
- This document is intended to provide information to ship's teams with regards to Casualty Clearing Organization procedures
- The information is not for disclosure outside of the RCN unless authorised by the document owner.
- Owners to review NDSODs Chapter 6 for more information

References

- Damage Control Vol. 1 (Sea);
- CFCD 102;
- Ship’s Standing Orders chap 3;
- Casualty Clearing Procedures Manual
- Military First aid;
- QSP Casualty Clearing Team Training; and
- Medical Equipment instructions manual.

References

- Damage Control Vol. 1 (Sea);
- CFCD 102;
- Ship’s Standing Orders chap 3;
- Casualty Clearing Procedures Manual
- Military First aid;
- QSP Casualty Clearing Team Training; and
- Medical Equipment instructions manual.

Objectives

PO 401 - Act as a member of a Ship's Casualty Clearing Team

- EO 401.1: Ship's Medical Organization;
- EO 401.2: CCT FA Review;
- EO 401.3: Establish Casualty Priority / Triage;
- EO 401.4: Communication, reporting and plotting;
- EO 401.5: Medical Equipment;
- EO 401.6: Respond to casualty pipe for Iso. Casualty;
- EO 401.7: Casualty associated with hazmat spill; and
- EO 401.8: Evacuate Casualty.

Schedule Day 1

- 0800 – Introduction
- 0820 - Ships Medical Organization
- 0850 – Break
- 0900 - Scene Size Up/Primary & Secondary Survey (Power Point, PP)
- 0955 - Break
- 1015 – Scene Size Up Primary & Secondary Survey (Practical/Pr Demo)
- 1100 – Break

Schedule Day 1

- 1110 - Bleeding/Burns (PP, Demo, Pr)
- 1150 – Lunch
- 1245 - Shock, Trouble Shooting (PP)
- 1315 - Chocking/CPR/AED (PP, Demo)
- 1400 – Break
- 1415 - Chocking/CPR/AED (Practical)
- 1530 - Home

Schedule Day 2

- 0800 - Splinting, Medical Equipment (PP, Demo)
- 0845 - Break
- 0900 - Splinting, Medical Equipment (PR)
- 0955 - Break
- 1015 - Establish Casualty Priority / Triage Power Point / Exercise
- 1050 - Break
- 1100 – Review and practice
- 1140 - Lunch

Schedule Day 2

- 1240 - Respond to casualty pipe and evacuate casualty to S.B. (PP, Demo, Pr)
- 1400 – Break
- 1410 - Review and practice previous two days.
- 1530 - Home

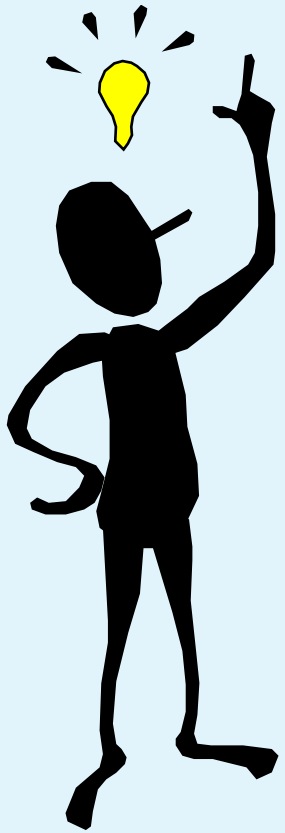
Schedule Day 3

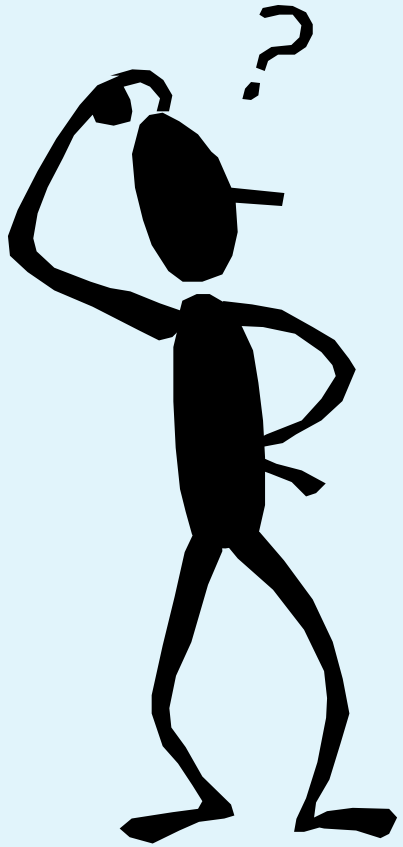
- 0800 - Communicating, Reporting and Plotting (PP)
- 0830 - Casualty associated with Hazmat spill (PP)
- 0850 - Break
- 0900 - Casualty associated with Hazmat spill (PR)
- 0955 - Break
- 1015 - Ships PA to schedule towards weakness
- 1130 - Lunch
- 1230 - PC Check, Act as a member of CC team

Schedule Day 3

- 1330 - Break
- 1340 - Secure and Clean equipment
- 1415 - Course critique
- 1430 - Home

Questions?



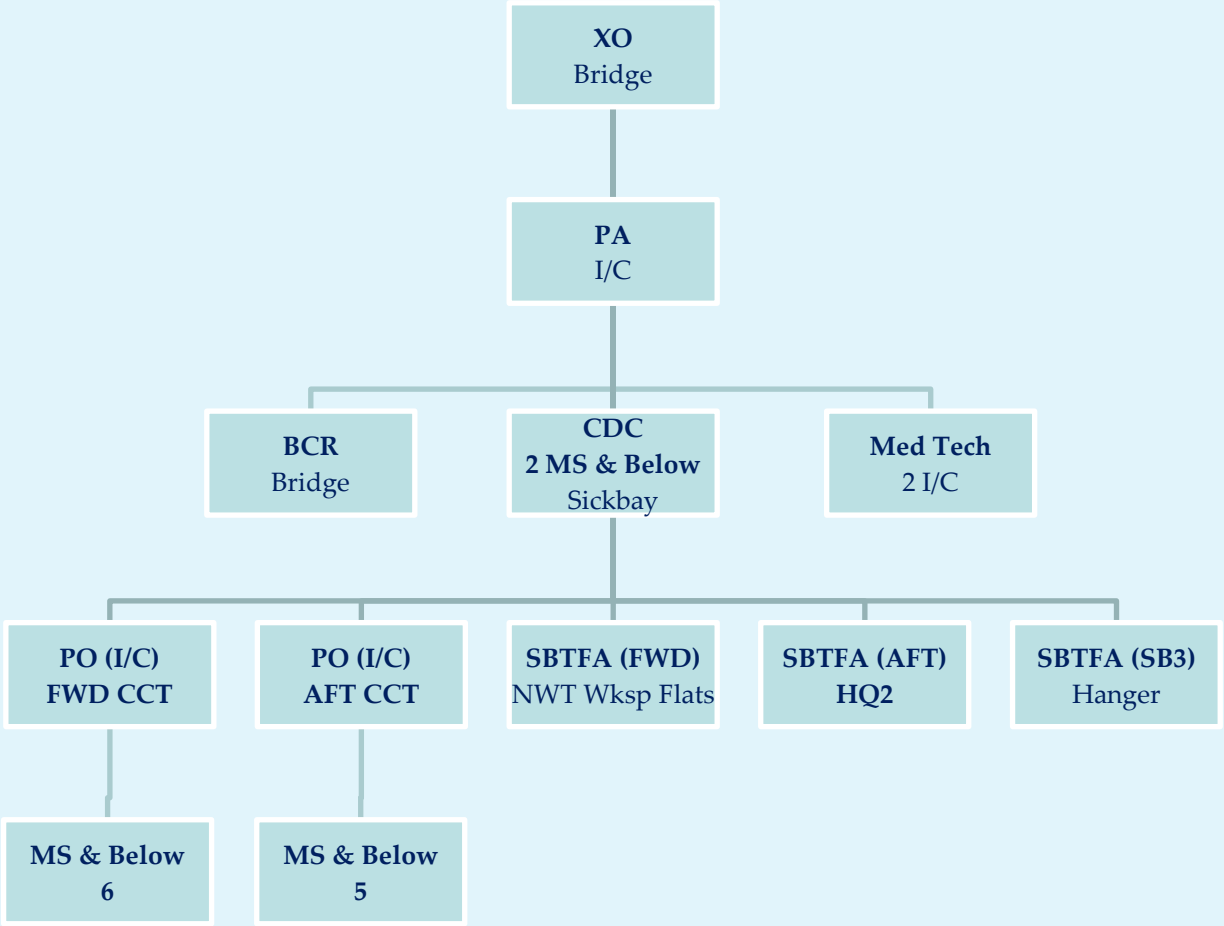


EO 401.1

0820 - 0850

Ships Medical Organization

Organization & Location (Halifax Class)



Duties & Responsibilities - XO

Responsible to the CO:

- training of CCT's; and
- guidance & orders for evacuation of casualties from the Ship.

Duties & Responsibilities - PA

Responsible to the CO:

- prevention of illness and dental fitness in the Ship;
- advise on the physical efficiency of the Ship's Company;
- diagnosis, care and treatment of sick or injured onboard;
- advice on the evacuation from the Ship of the sick or injured; and
- the security and release of medical documents and information on Ship's personnel.

Duties & Responsibilities - PA

Responsible to the XO:

- organization and administration of Sickbay;
- advice on matters of hygiene and sanitation;
- maintenance of adequate medical supplies on board;
- cleanliness and good order of the Sickbay;
- security of dangerous drugs, medicines and poisons; and
- assistance in the organization and training of the Ship's First Aid Teams.

Duties & Responsibilities - Med Tech

Responsible to the PA/MO:

- Action/Emergency Stations in FWD CCT;
- I/C of medical treatment of casualties within FWD CCT;
- re-assessment of casualty priority within their section (FWD CCT), can increase priority cannot decrease priority); and
- Isolated Casualty/Rescue Stations in Sickbay/Hanger.

Duties & Responsibilities - BCR

Responsible to the PA/CDC for:

- transmission of casualty data to the XO during action/Emergency stations;
- ensures XO is aware of movements of CCT relocations;
- ensures manning pools requirements are passed on to XO; and
- proper communication with Sickbay during Action/Emergency stations.

Duties & Responsibilities CDC

Responsible to the PA for:

- casualty accounting and communications during Action/Emergency stations to/from CCT's and BCR;
- preparation of casualty messages;
- closing up in Sickbay for isolated casualty, including casualty associated with HAZMAT spill; and
- closing up in the Hangar/Sickbay for Rescue Stations.

Duties & Responsibilities - CCT I/C's

Responsible to the PA for:

- acts as overall I/C for the operation of their respective CCT's;
- medical equipment in their CCT location;
- i/c of medical treatment of casualties within AFT CCT;
- re-assessment of casualty priority within their section (AFT CCT), can increase priority cannot decrease priority; and
- evacuation of casualties to Sickbay, including casualty associated with HAZMAT spill.

Duties & Responsibilities - CCT Members

Responsible to the CCT I/C for:

- their personal First Aid Kit;
- casualty priority allotment;
- adequate treatment to casualties; and
- responding to isolated casualty, including casualty associated with HAZMAT spill.

Duties & Responsibilities - SBTFA's

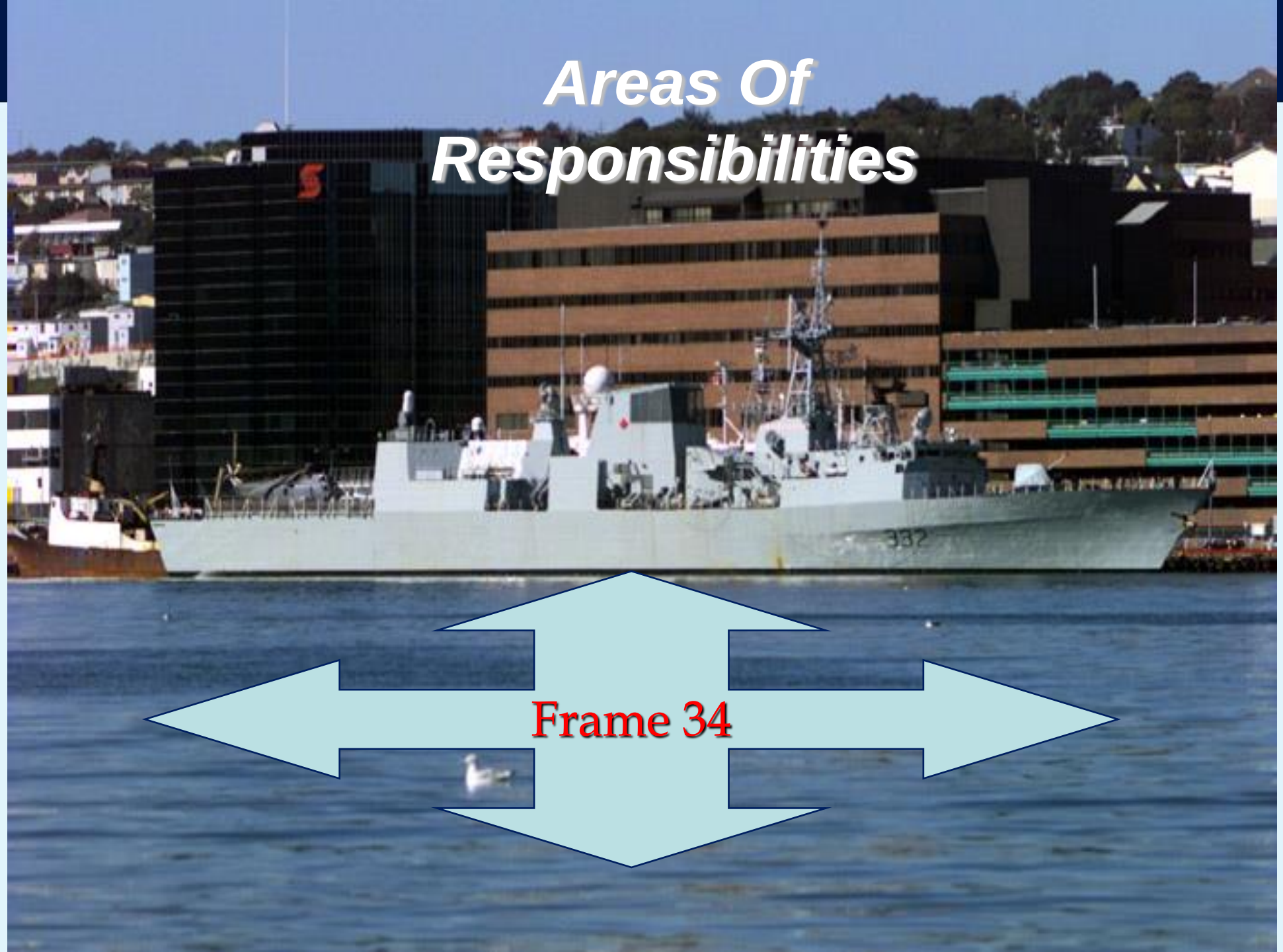
Responsible to the PA for:

- removal of casualties from hazardous areas and notifying their respective supporting CCT;
- immediate First Aid treatment of casualties in safe clearing area; and
- assist as required in multiple casualty or rescue situations when not required for DC.

ALT LOCATIONS HALIFAX CLASS

- Sickbay alternate in Wardroom of CO’s Cabin;
- Aft CC alternate in ADR Flats or Hangar;
- Fwd CCT alternate in CO’s Cabin or Hangar.

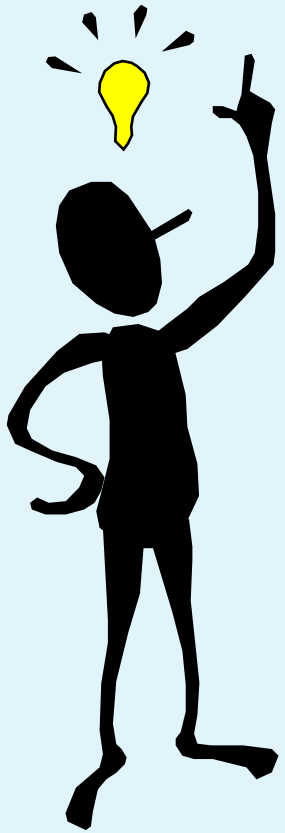
Areas Of Responsibilities

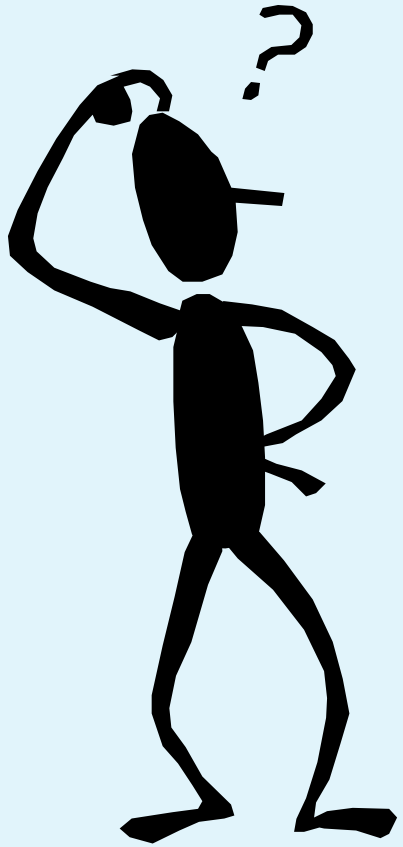


HFX AFT CASUALTY CLEARING

- 2 and 3 Deck aft of Frame 34 including;
- Hangar;
- Flight Deck;
- Laundry.

Questions?





CCT FA REVIEW

EO 401.2

Scene Size-Up

- Determine and ensure that the scene is safe;
- What was the mechanism of injury?;
- Determine the number of casualties; and
- Send for equipment/ medical aid.

Approaching the Casualty

- Identify yourself and tell the casualty not to move; and
- Universal Precautions: Make sure gloves are on.

Primary Survey

- **M** Massive Bleeding; (stop any massive bleeding)
- **A** Airway; (assess the airway)
- **R** Respirations; (check for breathing)
- **C** Circulation; (check for pulse)
- **H** Hypothermia; (cover patient to prevent heat loss)
- **E** Everything Else. (rapid body search)

Treat life threatening injuries as they present

Open and check Airway

- If the mechanism of injury suggests a head or spinal injury, manually stabilize c-spine;
- If unconscious and you suspect a head or spinal injury open the airway using the jaw-thrust without a head tilt. If no suspected head or spinal injury, open the airway using a head-tilt chin lift;
- Clear the airway of obstructions if required; and
- Insert oral airway if needed and trained.

Check for Breathing

- **Look, Listen and Feel** for breathing; Is it fast or slow?
- If there is no effective breathing, begin CPR.

Check for Circulation

- Check for a pulse; Is it fast or slow;
- Check Cap refill skin condition, colour; and
- If circulation is present, conduct a rapid body search to check for life threatening conditions.

Rapid Body Search

- The RBS should not take longer than 30 seconds;
- Neck to toe exam;
- Check the back;
- Conduct interventions for any life-threatening conditions as they are found; and
- If no life-threatening conditions were found, proceed with the secondary survey.

SECONDARY SURVEY

- Interview/ HX S.A.M.P.L.E., M.I.S.T.
- Assess LOC using AVPU
- Vitals
- Head-to-toe examination

Interview the Casualty or Bystanders

S - Sign and symptoms

What things are bothering you?

A - Allergies

Do you have any allergies?

M - Medications

What medications are you taking?

P - Past Medical History

What medical conditions do you have?

L - Last Meal

When did you eat last?

E - Events before the incident

What happened to cause the situation?

M.I.S.T.

- Mechanism of Injury;
- Injuries
- Signs (V.S.) and Symptoms
- Treatment

Assess the LOC using AVPU

Level of Response	Characteristic Behaviour
Alert	Eyes open, able to talk
Verbal	Responds to commands or questions
Painful	Facial grimace, moans or groans, withdraws from painful stimuli
Unresponsive	No response

Vital Signs

- Vital Signs are checked every 5 minutes in an unstable casualty and every 15 minutes in a stable casualty;
- Check the following:

Breathing

- In the secondary survey you are concerned with the **Rate, Rhythm and Quality** of breathing;
- **Look, Listen and Feel** for 15 seconds and multiply by 4 to give you an average in 1 min:
  - 12 – 20 per min. Normal
  - > 24 per min. Hyperventilation
  - < 10 per min. Hypoventilation

Pulse/Circulation

- Check the pulse for 15 seconds then multiply by 4 to get the rate in 1 minute:
  - +/- 70 per min. Normal
  - > 100 per min. Tachycardia
  - < 60 per min. Bradycardia

Skin Characteristics

- Check the skin color, temperature and cap refill.

Head-to-toe Examination

Conduct a through head-to-toe exam

Head

- Palpate skull for deformities;
- Check for bruising around the eyes (raccoon eyes), bruising behind ear (battle sign), and clear fluid from the ears or nose (cerebrospinal fluid);
- All are signs of a basal skull fracture (base of skull). Check pupils for equality and reactivity;

Head-to-toe Examination

Airway – Check and make sure airway is patent;

BCs – Recheck breathing and circulation as per vital signs time table;

Neck – Trachea midline or deviated? Jugular vein distension? (signs of tension pneumothorax)

Head-to-toe Examination

- **Inspect for DCAP -** deformities, contusions, abrasions and penetrations;
- **Palpate for TIC -** (tenderness, instability, crepitus);
- **Chest -** Inspect for DCAP, palpate for TIC;
- **Continue** Inspecting DCAP and palpating for TIC for the abdomen, pelvis, lower and upper extremities. If you forgot the back on the primary survey do it now;

ONGOING CASUALTY CARE

- Constantly talk to your casualty, reassess the ABCs;
- Record what you have observed and what treatment you performed for the casualty;
- Make the casualty as comfortable as possible; and
- If you applied bandages, splints or oxygen, ensure you check them as well.

SCENE SIZE UP AND PRIMARY/SECONDAY SURVEY PRACTICAL

1015 TO 1100

BLEEDING/BURNS PP/DEMO/PRACTICAL

1110 - 1150

BLEEDING

- Apply direct pressure;
- Use indirect pressure if needed (*i.e.* pressure points);
- Tourniquet if needed and if trained in its use;
- Properly applied pressure dressing;
- Raise and rest injured limb;

BLEEDING

- Apply new dressing if soaked through, DO NOT remove soaked dressings;
- DO NOT remove imbedded objects;
- If object is imbedded, pad both sides and then bandage in position found; and
- Seek medical aid.

BURNS

To estimate burn size, use the Rules of Nine:

- Head and neck 9%
- Posterior trunk 18%
- Anterior trunk 18%
- Each upper extremity 9%
- Each lower extremity 18%
- Genital 1%

Patient palm can be used to determine percentage of burns. 1 patient palm = 1%

BURNS

- For smaller burns (less than 10% body surface burn) cool burned area with running water for 20 minutes;
- For larger wounds use a clean blanket and keep patient warm as hypothermia may occur;
- Remove watch and or jewellery;
- Remove hot/wet clothing;

BURNS

- If patient is pale, rest lying down;
- Once area is cooled, apply dry dressing and bandage loosely in place;
- DO NOT APPLY CREAMS; and
- Burns to the face, neck or groin MUST be seen by medical aid/sickbay.

SHOCK AND TROUBLE SHOOTING (PP)

1245 TO 1315

SHOCK

- Shock is the result of inadequate organ perfusion and tissue oxygenation;
- Haemorrhage is the most common cause in trauma patients;
- Any injured patient who is cool and has tachycardia is considered to be in shock until proven otherwise;

SHOCK

- Management of shock should include early identification and control of the source of haemorrhage; and
- Administer oxygen, control external bleeding and elevated lower extremities, keep patient warm and give fluids by mouth if patient is conscious.

TROUBLESHOOTING CHART

- Treat for Shock by treating the injury causing it;
- Recheck dressings frequently;
- Treat the entire casualty and do not get tunnel vision towards the injury/illness;
- Continuously monitor CABs, if you move a casualty you have to recheck the CABs;
- If your casualty is deteriorating you might have missed something. Go back and perform another

  primary survey;

TROUBLESHOOTING CHART

- Talk to your casualty, it helps reassure them as well as others;
- Always consider the environment in which your casualty is located and assist in maintaining body temperature;
- DO NOT go beyond what you are trained to do; and
- Never abandon your casualty.

CHOCKING/CPR/AED (PP/DEMO)

1315 - 1400

CHOKING – PARTIAL

- If the person is able to breathe, cough, speak and cry effectively;
- Reassure and stand-by;
- Encourage to cough and breathe;
- Observe for changes in casualty;

PARTIAL

- If casualty deteriorates, activate the emergency response and attempt to remove the obstruction; and
- If the obstruction is caused by an allergic reaction, assist with Epi-Pen and seek medical aid.

Chocking Complete/Conscious Adult/Child

- If the person is unable to breathe, cough, speak or cry effectively, activate emergency response;
- Conduct abdominal thrust until effective or casualty is unresponsive. (chest thrusts for those who are pregnant or in a wheel chair)

Chocking Complete Conscious Infant

- 5 back blows followed by 5 chest compressions until effective or infant becomes unresponsive

Unconscious Complete/Adult, Child and Infant

- Activate emergency response;
- 30 Compressions;
- Open the airway;
- Remove foreign body only if seen;
- 2 attempts to ventilate;

Unconscious Complete/Adult, Child and Infant

- Landmark as for CPR and conduct 30 compressions;
- Adult: Compress at least 2 inches; Child: About 2 inches; Infant: At least 1.5 inches (4 cm), and
- Repeat until ventilation is successful.

CPR CHILD/ADULT

- Recognition: unresponsive, no breathing or no normal breathing; (*i.e.* only gasping)
- Activate emergency response and send for AED;
- No pulse palpated within 10 sec;
- AMFR 2 and PHTLS only, Standard FA do not do pulse check;

CPR CHILD/ADULT

- CPR Sequence; (C A B)
- Begin compressions first by land marking;
- child heel of hand on lower half of Sternum;
- Adult heel of hand on lower half of Sternum with second hand placed over first;

CPR CHILD/ADULT

- Compression Rate: At least 100/min;
- Compression depth: Child about 2 in. (5 cm);
  Adult at least 2 in.(5 cm);

CPR CHILD/ADULT

- Airway: Head tilt and chin lift;
- Breathing: 2 breaths in one second;
- Compression/Ventilation Ratio: 30:2;
- 5 sets/2 mins; and
- Defibrillation: Attach and use AED ASAP.

- CHOCKING/CPR/AED PRACTICAL

1415 TO 1530

- SPLINTING AND MEDICAL EQUIPMENT (PP, DEMO)

0800 TO 0845

SPLINTING

- Steady and support the injured limb;
- Fully expose and assess the injured limb;
- Assess pulse, cap refill, sensation, motor function skin colour and condition in the injured limb;
- Compare to the other side;
- Dress any open wounds;

SPLINTING

- Apply ice pack for 20 mins every two hrs;
- Do not apply sager splint to joint injuries;
- Using available materials, splint the injured limb in the position found;
- Sling upper extremities; and
- Assess circulation after immobilization is completed and transport to medical aid.

MEDICAL EQUIPMENT

EO 401.5

CCT EQUIPMENT

Within CCT location

- O2 resuscitator;
- O2 bottles;
- First Aid Kits;
- Medical Panniers;

CCT EQUIPMENT Con't

- KED;
- Hard Collars;
- Blankets; and
- Damage Control Board.
- Sager splint - Only available in Sickbay & Alt Sickbay.

Medical Panniers

- Used when the Ship:
  - in Action / Emergency Station;
  - during SAR or;
  - in multi casualty situation;
- Provides supplies to CCT;
- Consists of 2 chests; and
- CCT I/C's are responsible for accounting and replenishment.

Equipment Through Out Ship

- Ashton-Waters stretcher;
- First aid bags on stretchers;
- CCT replenishing boxes;
- Mass casualty panniers;
- water gel blankets; and
- Med Evac 2 (Stokes litters X 2)

First Aid Bags Through Out Ship (Kit List)

- Triangular bandage 7ea;
- Field dressing 5ea;
- Splint universal 1ea;
- Gloves 5pr;
- Water gel dressing small 2ea;
- Water gel dressing large 2ea;
- Elastoplast dressing 1bx;
- Sterile 4X4 or pressure bandage 4env;
- Scissors universal 1ea; and
- Resuscitator mouth to mouth 1ea.

First Aid Boxes-Ship Mounted (Kit List)

- Triangular bandage 35ea;
- Field dressing 25ea;
- Splint universal 5ea;
- Gloves 20pr;
- Water gel dressing small 10ea;
- Water gel dressing large 10ea;
- Elastoplast dressing 5bx;
- Sterile 4X4 or pressure bandage 20env;
- Scissors universal 1ea;
- Resuscitator mouth to mouth 5ea; and
- Aqueous chlorhexidine solution 1bt.

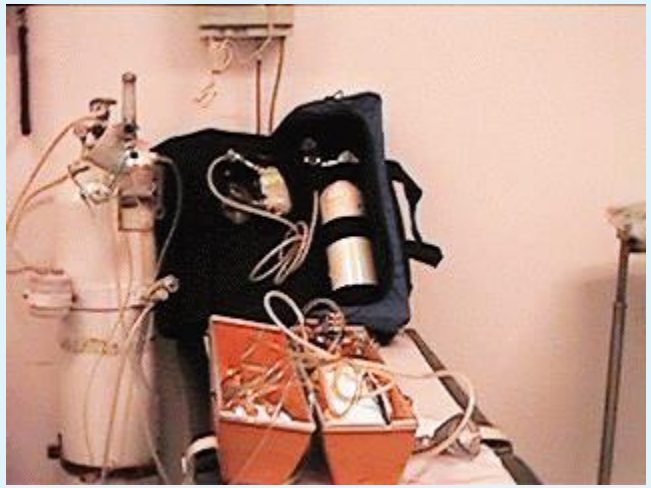
OUTLINE

Power Point & Demonstration

- Oxygen Therapy
- Perform Vital Signs
- PROPAK
- Zoll and AED
- IV Preparation
- KED
- Sager
- Ashton water Stretcher

OXYGEN THERAPY

- Room air: 21% O2
- AR by Mask: 16% O2
- Nasal Cannula: 28% O2
- Simple Mask: 40% O2
- Non Re-breathable Mask: 80 – 90% O2
- BMV: 90% - 100% O2



VITAL SIGNS

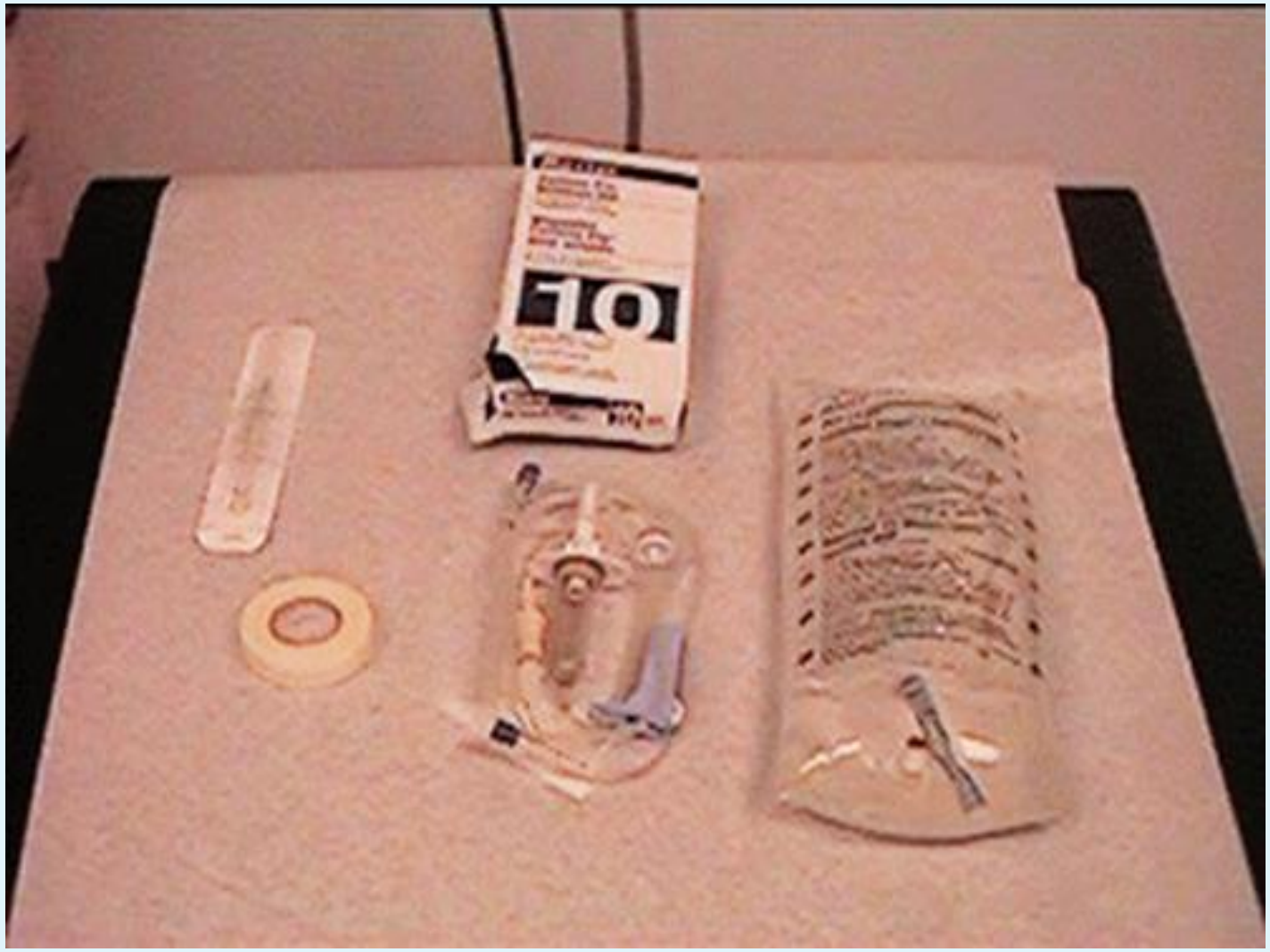
- B/P (N) ?
- P (N) ?
- R (N) ?
- T (N) ?
- SPO2 (N) ?

EQUIPMENT

- PROPAK - For monitoring V.S and cardiac rhythms.
- ZOLL - For monitoring V.S., Cardiac rhythms, Defibrillating, Pacing, and Cardio version.
- AED- Automatic External Defibrillator

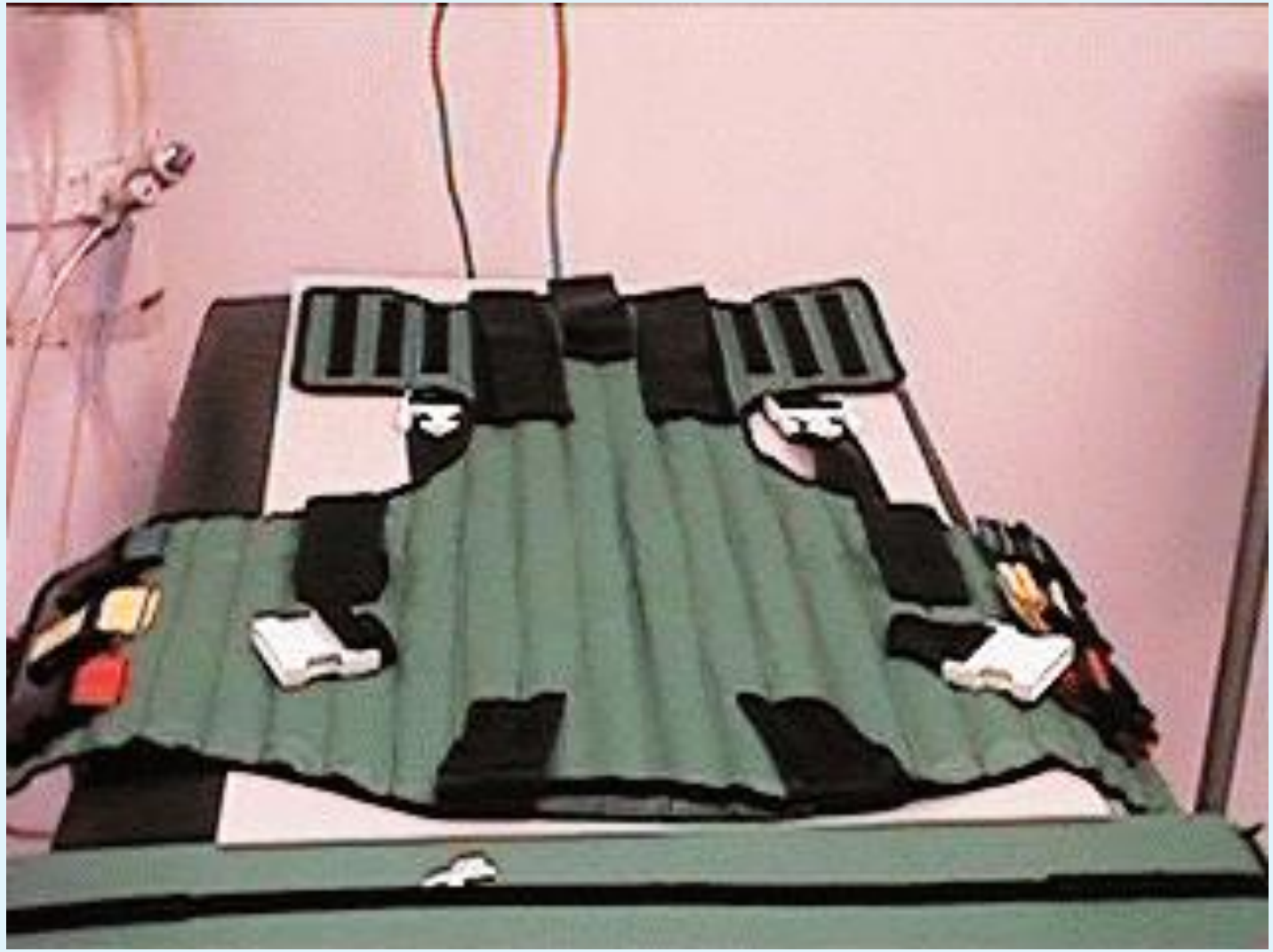
IV Preparation

- Demonstration
- Types of Solutions
- IV Catheters



KED

- Demonstration
- C-Spine
- Pelvic Fracture



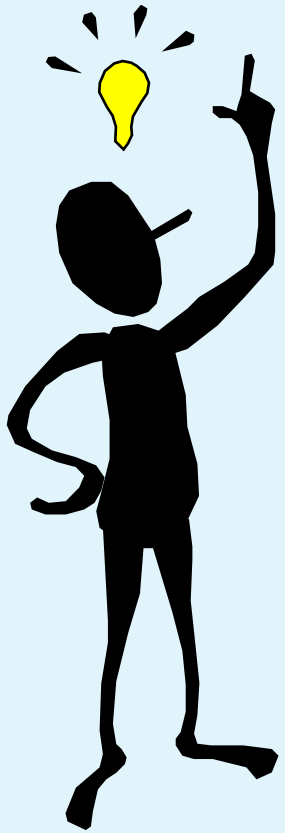
SAGER SPLINT

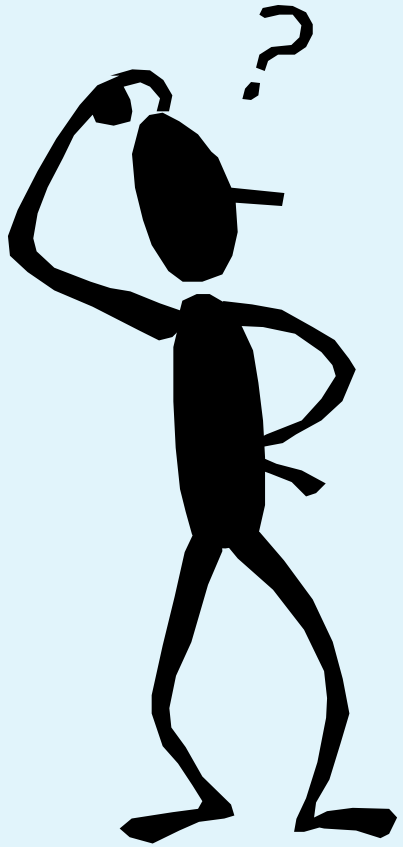
- Demonstration
- Single or Bi-lat Fractures
- Not for Joints

ASHTON WATERS STRETCHER

- Demonstration
- Head straps obsolete

Questions?





SPLINTING AND MEDICAL EQUIPMENT (PR)

0900 TO 0955

ESTABLISH CASUALTY PRIORITY AND TRIAGE

(PP, EXERCISE)

EO 401.3

Triage

What is triage?

- The process of sorting and classifying casualties by assigning priorities for first aid and transportation;
- This is done when the number of casualties outnumber first aiders;
- This should be done ASAP sorting them into 4 levels.

Triage

If you are in charge proceed as follows:

- Scene survey – make sure scene is safe;
- Scene survey - # of casualties;
- Go to the nearest casualty;
- Check LOC, ABC's;
- FA only for life-threatening conditions;

Triage

Ensure the following:

- Airway
- Breathing
- Circulation

- If necessary put in recovery position;
- Prioritize accordingly and mark on body; and
- Repeat this process with the next nearest casualty.

Triage

- Mechanism of injury can give you clues as to which casualty should go first;
- Remember when you come across a casualty with a suspected spinal cord injury you MUST establish c-spine control at the time you start you initial assessment;

PRIORITIES

- Do not delay transport to give first aid for minor conditions;
- The casualties of the highest priority should be ansported first; and
- Give ongoing casualty care to each casualty.

PRIORITIES

Priorities

- Priority 1 - **Immediate:** ASAP off Ship;
- Priority 2 - **Early:** Within four hours;
- Priority 3 - **Routine:** No evacuation needed to holding then 24 to 48 hrs or longer; and
- Priority 4 - **Deferred:** So serious that the probability of survival is remote.

PRIORITY 1 (Highest)

- Given to:
  - Unconscious casualty
  - Airway difficulties;
  - Respirations <10 or > 29 min;
  - Pulse > 120;
- Given to casualties requiring life saving medical intervention;
- Roughly 20% of all casualties will be Pri 1;

Priority 1 (Immediate)

- Airway Obstruction;
- Nonresponsive;
- Sucking chest wound;
- Severe shock; and
- Severe bleeding.

Priority 2 (Early)

- Have serious, possibly life threatening injuries but are temporarily stable;
- Good probability of survival;
- Condition not compromised by short delay of treatment;

Priority 2 (Early)

- Abdominal wounds with stable VS;
- Open or complex fractures; and
- Second or third degree burns to less than 10% of the body, minus face neck or groin.

Priority 3 (Routine)

- Usually walking wounded;
- Facial or jaw injuries without respiratory distress;
- Combat stress problems;
- Lesser fractures;
- Minor burns; and
- Approx 40% of casualties will be Pri 3.

Priority 4 (Deferred)

- So serious that the probability of survival is remote. (Casualties in this group should be moved to the side in a quiet area and be prepared to provide as much comfort as possible);
- Approximately 20 % of casualties are priority 4;

Priority 4

- Normally casualties with no pulse are placed in this group; and
- One exception is those casualties with no pulse as a result of lightning strike should be given Pri 1.

Dealing With Death

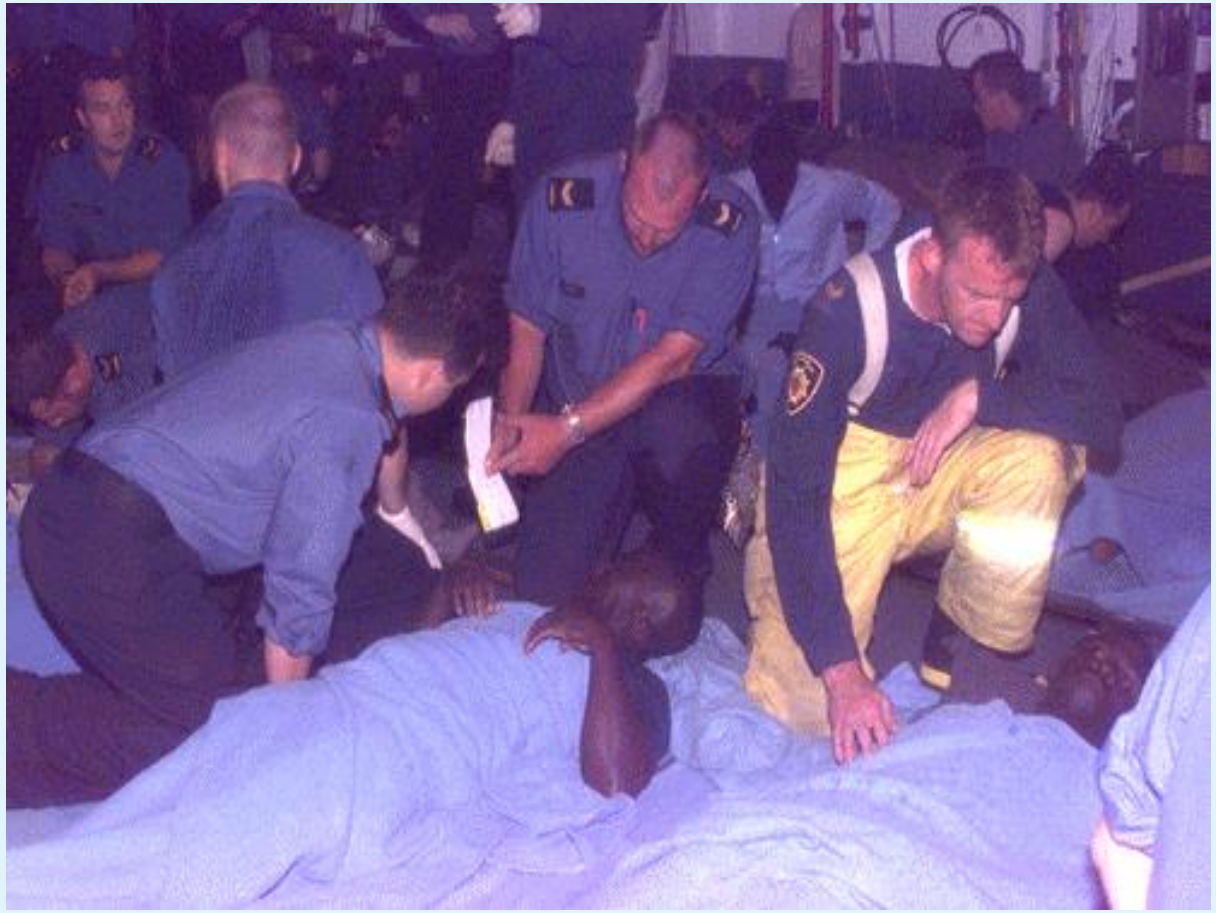
- If death is imminent, be compassionate and dignified;
- Record any dying statements or requests and make a note of the circumstances of death; and
- Protect personal belongings.

Changing Priorities

- When the patients condition changes (for the better or worse) it may be necessary to change the assigned priority.
- This should be done in consultation with the Ship's PA.
- Keep in mind that changes for the worse may mean you have missed something, so go back and check again.

Triage Tags

- Triage tags are a means of identifying casualties and recording vital information regarding their medical condition and any changes that may occur.



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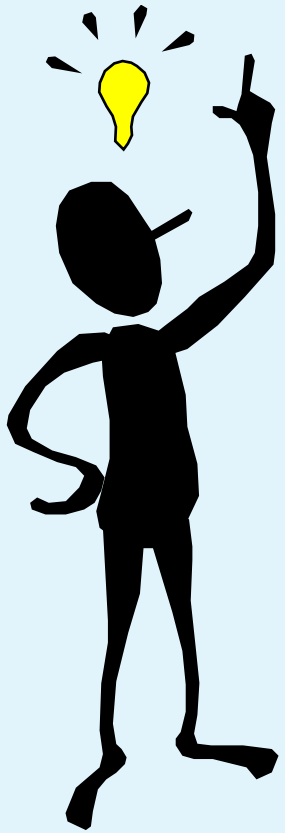
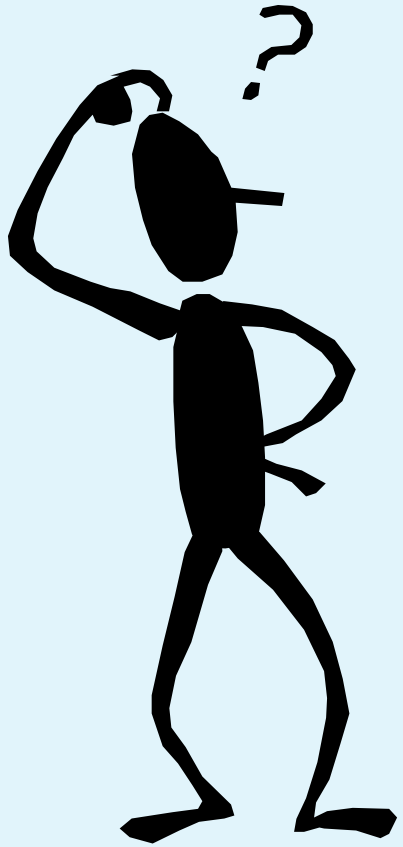




Respond to Casualty pipe for Isolated Casualty and Evacuate Casualty to Sickbay
(PP, DEMO, PR)

1240 to 1400

Respond to Casualty pipe for Isolated Casualty
EO 401.6

General

The following personnel MUST respond to any isolated casualty pipe:

- PA
- Med Tech
- CCT members
- POOW/Duty Cox'n

Note: XO and Coxn should respond if operationally feasible.

Equipment Brought To Scene By CCT

- First Aid Kit
- Cervical collar
- Oxygen resuscitator; and
- Ashton-Waters stretcher

Note: if additional equipment is required, the CCT I/C will direct personnel to muster specific medical equipment.

Isolated Casualty While At Sea

Discoverer's Action:

- Assess hazards at the scene: make area safe for self and others;
- Call for HELP: Casualty, Casualty, Casualty in (Location);
- Assess responsiveness;
- Assess: M.A.R.C.H.E;
- Monitor; ABC's until Casualty Clearers arrive at scene; and
- Brief Casualty Clearer I/C of Tmt and CCT I/C.

Note: Do not leave the scene until secured by CCT I/C

CCT I/C Action

- Take **CHARGE** of Scene, receive briefing from the discoverer;
- Assign personnel: Assign CC Tmt I/C (until relieved by PA) and assisting members to provide treatment to casualty;
- Designate communication I/C, evacuation I/C, recorder; and
- Monitor Scene, I/C's, and recorder.

Note: Do not leave Sickbay until secured by MO/PA

Evacuation I/C Action

- Organize: crowd control, manpower and casualty transfer
- Brief: evac pers on route & assignments
- Notify and brief: CCT I/C on evacuation route
- Equipment: ensure all eqpt is returned

Communication I/C Action

Available Com's: Shincom, PRC and Runner

- Keep CCT I/C informed;
- CDC must be advised;
- CDC and Command must know casualties name and or position and priority;
- Through CDC request from command that the route be put out of bounds; and
- Insure CDC has informed command that casualty has arrived at Sickbay.

Recorder

- Date, Time, Name, Position
- Treatment
- A.V.P.U.
- M.I.S.T.
- S.A.M.P.L.E.
- V.S.
- Diagnosis

Sickbay Personnel Action

- Organize: the S.B. to receive the casualty
- Record: continue to record Tmt, V.S. etc
- Notify: command of any changes of casualties condition

Note

- The sequence of events for Isolated Casualty

  does not change while alongside during normal working hours. The only difference is in communication.
- The CCT I/C Action and Sickbay will inform the OOD vice OOW and if directed by the PA, the CDC will call for assistance from 911. The OOD will assign personnel to escort First Responders and Ambulance Crew to the proper location

Alongside after Normal Working Hours

An ISOCAS after working hours is the responsibility of the duty watch under the direction of the OOD.

Duties and Responsibilities - OOD

- Responsible to the Co for the care and handling of the casualty
- Part of the RRT
- Delegate a member of the duty watch to take charge of the casualty, if a casualty clearer is on board they will usually take charge of the treatment of the casualty
- Control the scene through the DCOXN

Duties and Responsibilities - OOD

- Ensure emergency equipment is brought to the scene;
- Responsible for the safe evacuation of the casualty to the jetty
- Ensure the QM has made the required phone calls (Ambulance/MP's); and
- Prepare a written report for CO, XO and PA

Duties and Responsibilities - DCOX'N

Responsible to the OOD for the following:

- Scene management, (personnel and equipment);
- Preparation of the evacuation route to the jetty;
- Possible requirement to help with first aid and or evacuation of casualty; and
- A detailed entry in the DCOXN log about the incident, first aid treatment and evacuation fro the Ship.

Duties and Responsibilities

Off Watch Personnel

Responsible to the DCOXN for the following:

- Part of the RRT
- Administering first aid;
- Preparing stretcher for use; and
- Acting as a stretcher bearer as required.

Duties and Responsibilities

Off Watch Personnel

Responsible to the DCOXN for the following:

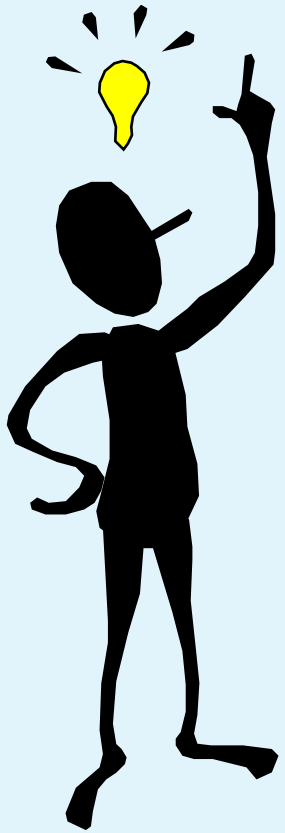
- Part of the RRT
- Administering first aid;
- Preparing stretcher for use; and
- Acting as a stretcher bearer as required.

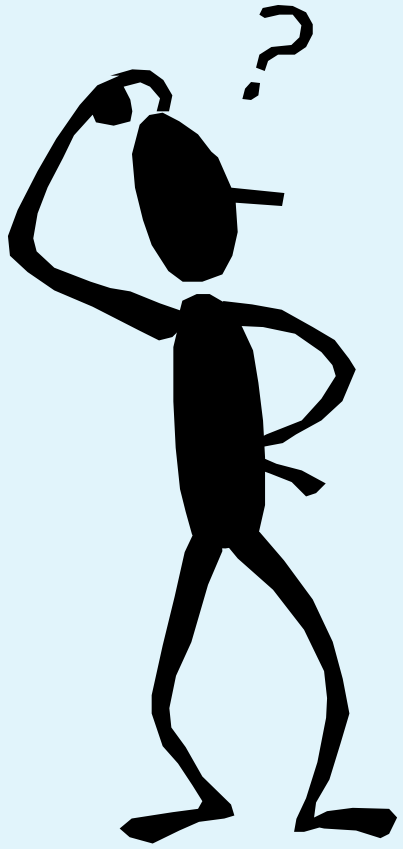
Duties and Responsibilities

Notes

- 1. The member in charge of treatment MUST brief the ambulance attendant on the condition and first aid given to the casualty.
- 2. All after normal working hour casualties must been seen by an MO or Civilian Doctor regardless of the severity of the injury.
- 3. If in the opinion of whomever is in charge the casualty, the movement would aggravate the condition then the duty watch must wait for the arrival of the Fire Department for further advice on treatment and movement of the casualty.

Questions?





EVACUATION OF CASUALTIES TO SICKBAY

EO 401.8

EVACUATION BEGINS EARLY

EVAC ROUTES

Evacuation Routes

- CCT member must be able to evacuate casualties SAFELY from the point of injury to:
- Safer area for primary treatment;
- CCT station/location;

Evacuation Routes

- The secondary location;
- Sickbay;
- Flight deck/hangar; and
- Upper decks.

Evacuation Routes Notes

- Quickest route not always the safest.
- Safest route not always possible.

EVACUATION ROUTES

- SBT’s are best qualified to direct teams to the best and safest route.
- Pri 3 casualty are able to assist in evacuation of more seriously injured casualties.

Hatches Without Sentries

- Check for heat, cold, condensation;
- Open away from yourself; and
- Slightest doubt, check with CDC or SBT.

MONITORING THE CASUALTY

- Casualty must be monitored at all times;
- Stop, reassess before and after going up/down a ladder, always ABC's;
- Ensure casualties safety, straps secure before movement; and
- Ensure equipment is secure.

MOVING THROUGH DOORS

- 4 pers through ahead of the stretcher, 2 on each side of passage and then pass the foot of the stretcher through;
- Feed the stretcher through and repeat process next door; and
- Minimum 4 pers lifting stretcher with one person assigned to monitor patient.

EVAC HAS MANY WAYS

LADDERS

- Minimum of 4 pers up and 4 down;
- No one on ladder when moving casualty; and
- Ensure equipment safe and will not injure patient.

CAUTION EVEN WHEN LEAVING THE SHIP

REVIEW

1410 TO 1530

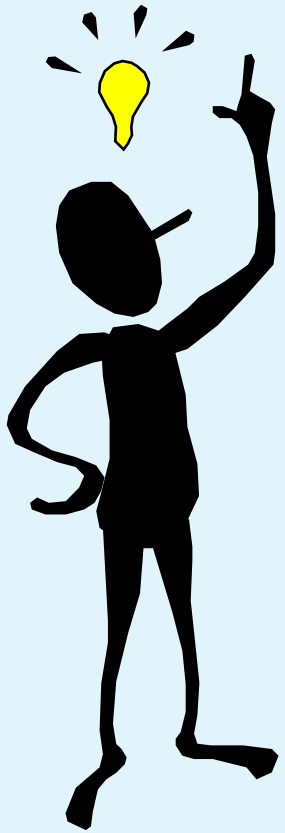
Go over material and practice previous two days

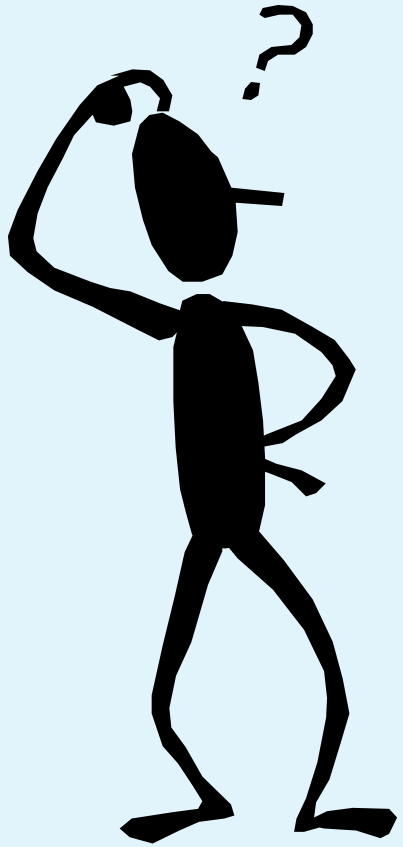
COMMUNICATION, REPORTING AND PLOTTING

EO 401.3

0800 TO 0830

Questions?





COMMUNICATIONS

AVAILABLE COMMUNICATIONS - HALIFAX CLASS

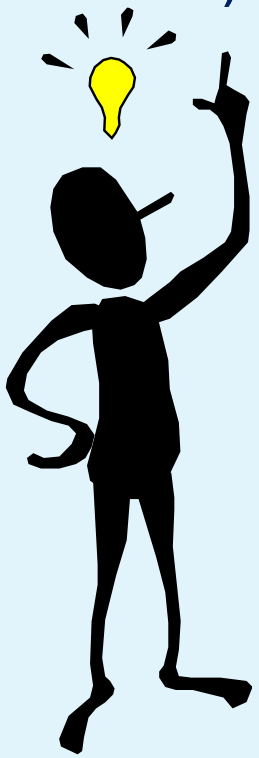
- Shincom
- Meet Me Net (56)
- Runner
- PRC's

Communication Notes

- Command must know; and
- CDC must be advised.

YOU MUST REPORT

- Casualties Name or Position
- Priority
- Location (injury location, current location if casualty has been moved)



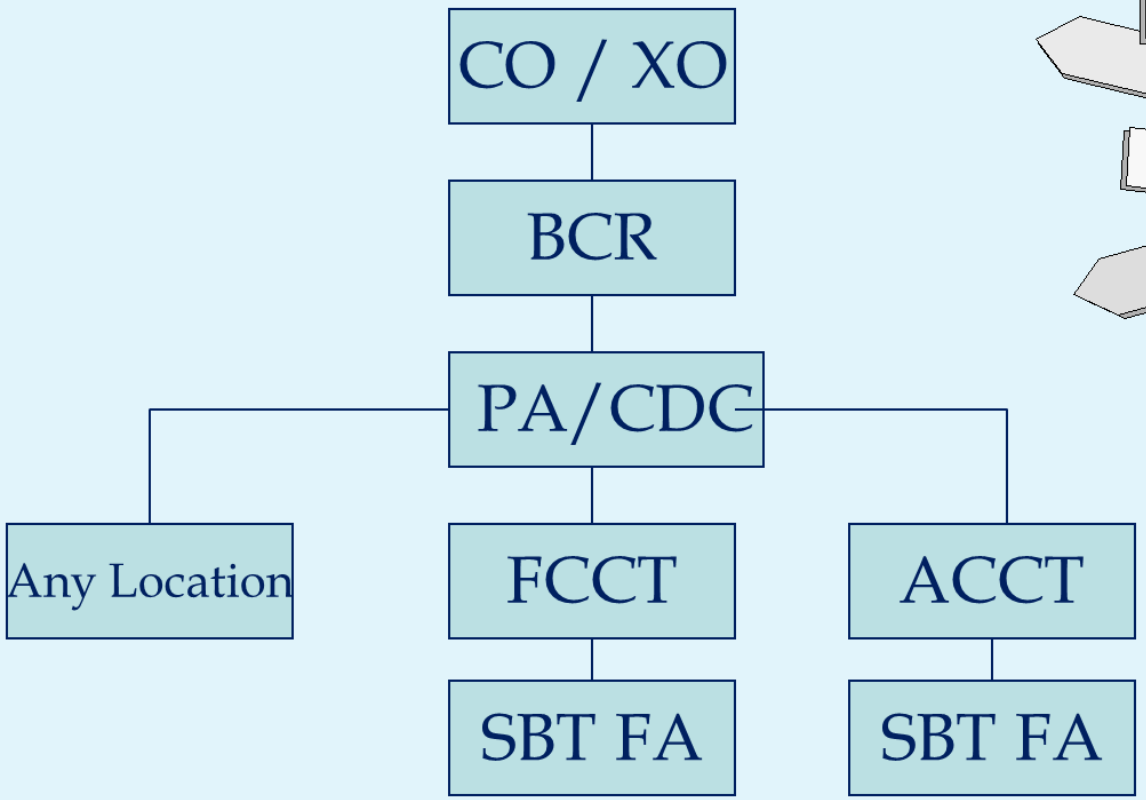
Communication Notes

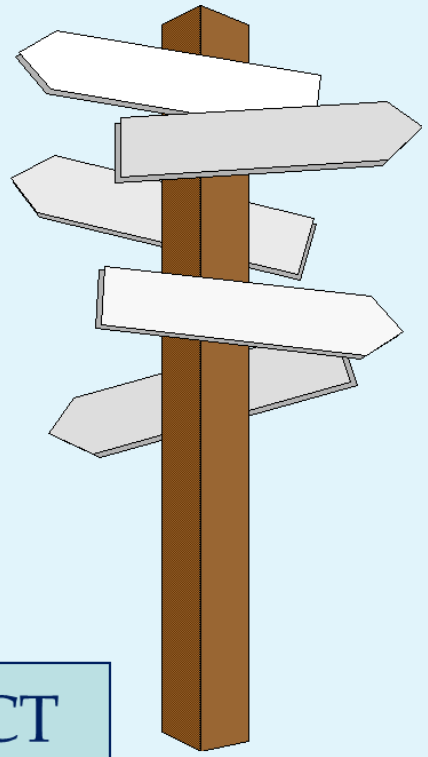
- Command
- HQ1
- Sickbay
- SBT I/C
- CCT I/C
- Command may order that a station to be abandoned and re-establish at their secondary location due to DC problems.

IMPORTANT

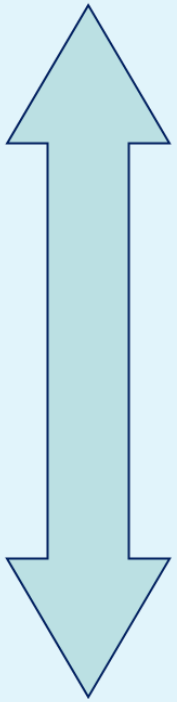
- It is imperative that Command and Sickbay be made aware of functional secondary location.

Communication Chain of Command





INFORMATION



FLOWS BOTH WAYS.

UP

&

DOWN

“Remember brevity and clarity are the best rules, give only the basic info req’d in a clear & concise manner, if more info is req’d for evaluation, it will be requested”

CCT Communicator Aide Memoire

CCT Aid Memoire

1. Establish comms

NOTE: use this time to rectify comms problem. “Sickbay, Aft/Fwd Casualty Clearing, comms check”

2. Report closed up when your Team is able to handle casualties, not only when members are present. “Sickbay, Aft/Fwd Casualty Clearing, ACCT/FCCT closed up minus2”

3. Report casualty arrival, ie. Name/position status (pri) “Sickbay, ACCT/FCCT, casualty LS Bloggins, arrived our location, Pri2”

4. Report any casualty transfers, prior to commencement unless you are evacuating for safety reasons. “sickbay, ACCT/FCCT, casualty LS Bloggins is being transferred to sickbay now”

CCT Aide Memoire

5. Unless notified otherwise, communicate only through your Chain of Command.
6. Prior to talking on the Net, listen, if someone is speaking, WAIT. If your situation is more critical then BREAK IN on the Net “break, break, break” sickbay, this is ACCT/FCCT.....’
7. After you are secured by sickbay, cease comms, but wait for acknowledgement. “sickbay, ACCT/FCCT, OUT”

“THERE IS NO SITUATION WHERE CASUALTY OR GROUP OF CASUALTIES ARE MORE IMPORTANT THAN THE INTEGRITY OF THE SHIP”

COMMUNICATION CLOSE UP

PLOTTING

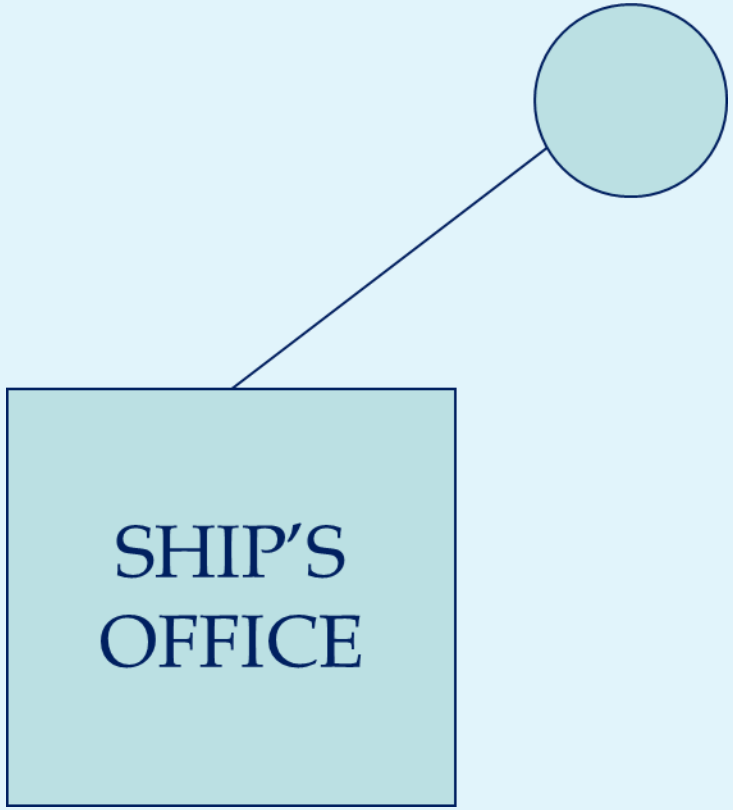
USING DC BOARD

PLOTTING

- LINE FROM THE SECTION WHERE CASUALTY IS REPORTED

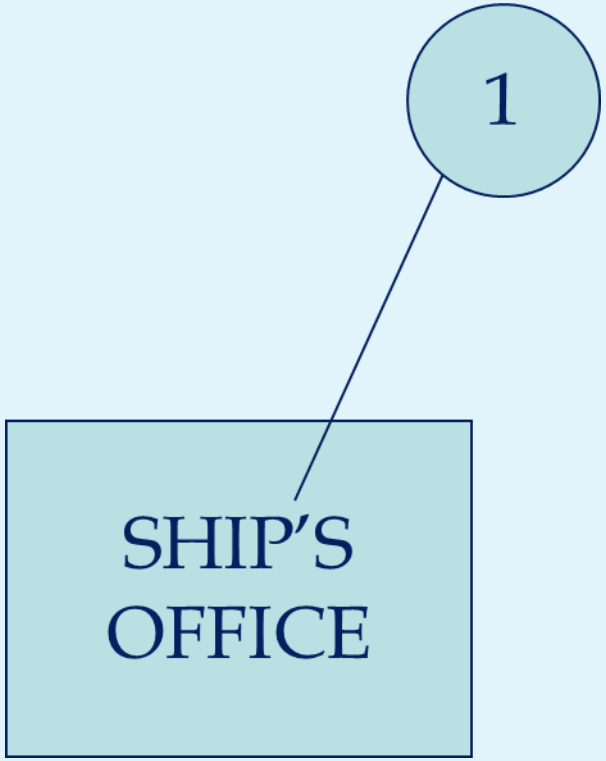
PLOTTING

- ADD A CIRCLE TO THE END OF THE LINE



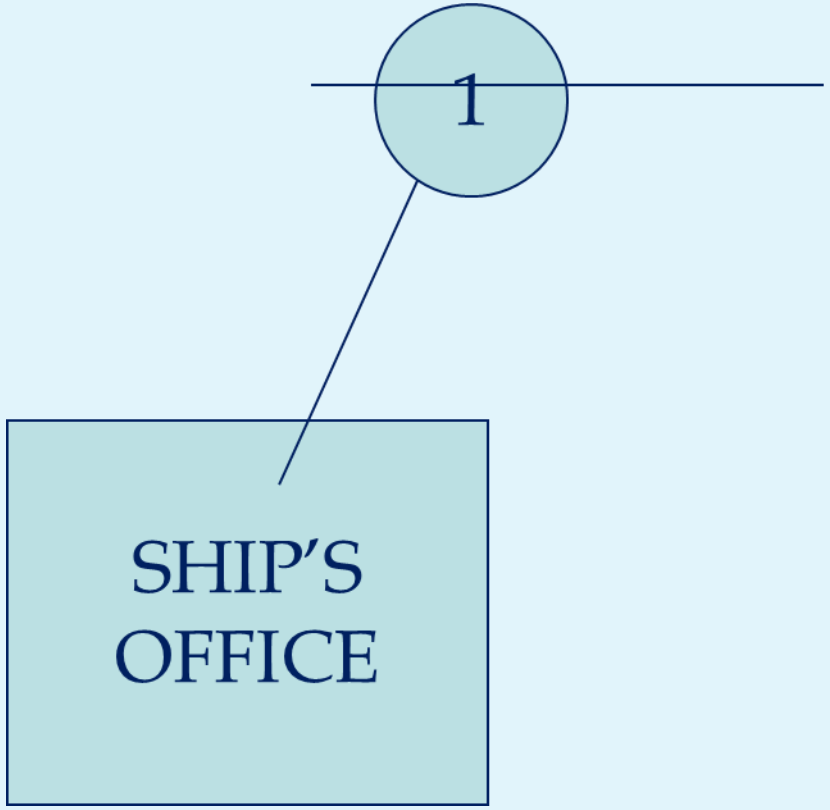
PLOTTING

- REQUEST NUMBER FROM SICKBAY
- CDC WILL GIVE YOU NEXT # IN CASUALTY LOG



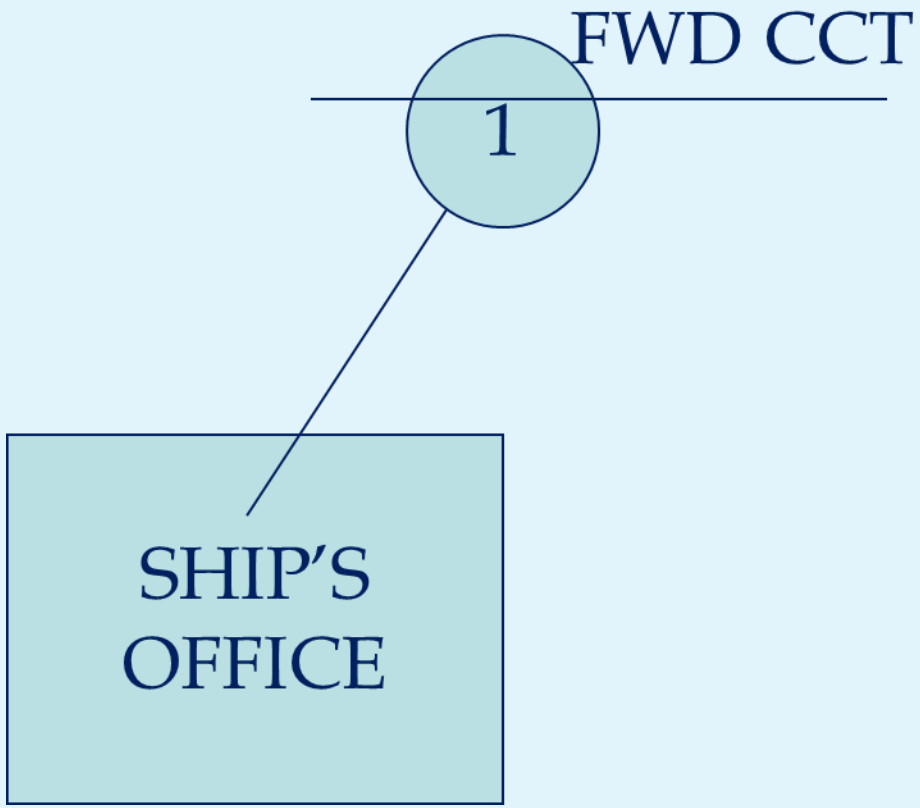
PLOTTING

- FIRST AID TEAM DISPATCHED
- CC TEAM NUMBER AND TIME DISPATCHED



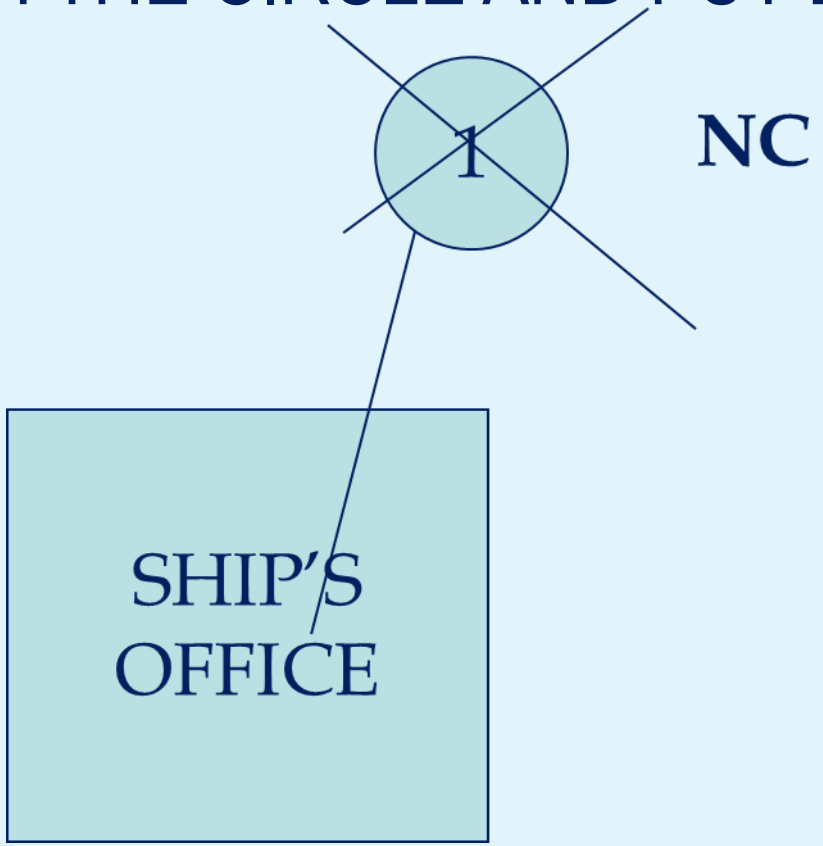
PLOTTING

- IDENTIFY WHEN THE CASUALTY IS IN CCT LOCATION
- FWD
- AFT



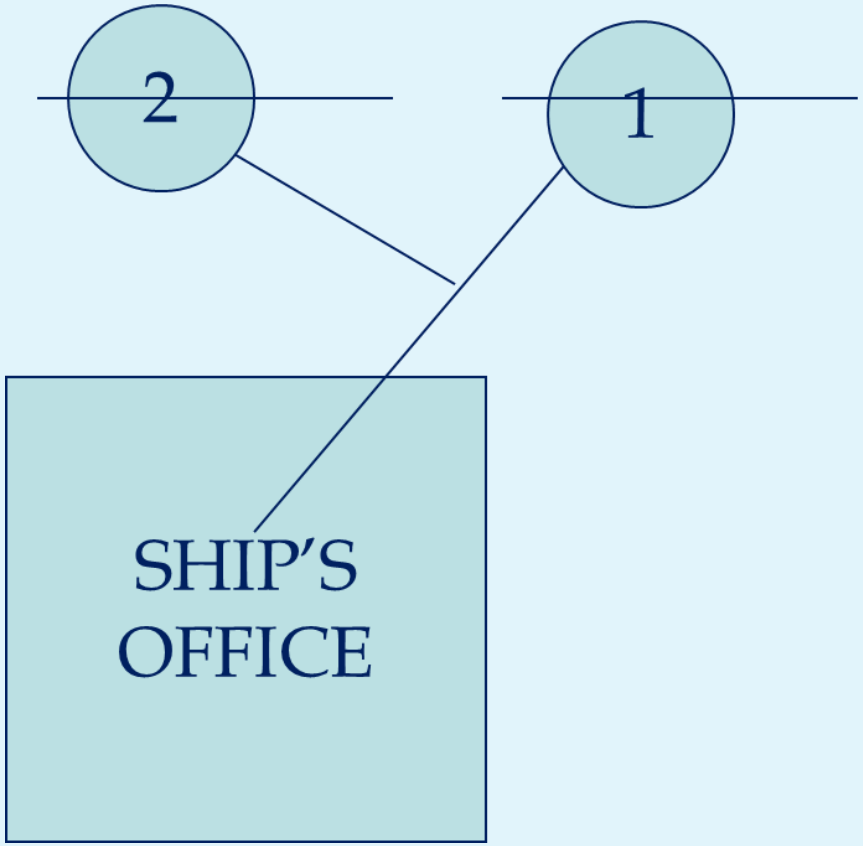
IF NO CASUALTY IS FOUND IN STATED LOCATION

- PUT AN X THROUGH THE CIRCLE AND PUT LETTERS (NC).



MORE THEN ONE

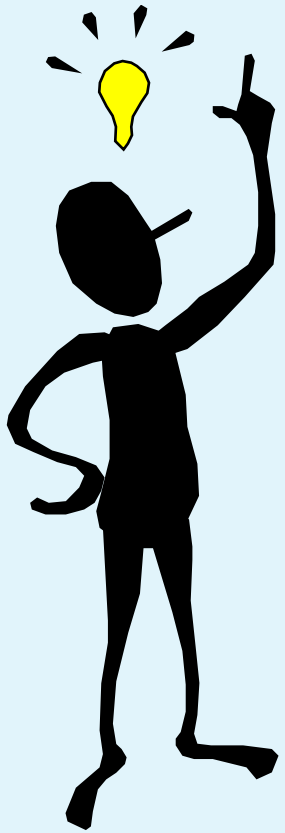
- IF MORE THEN ONE CASUALTY IN IS FOUND IN THE SAME LOCATION GET ANOTHER CASUALTY # AND PLOT AS PREVIOUS.

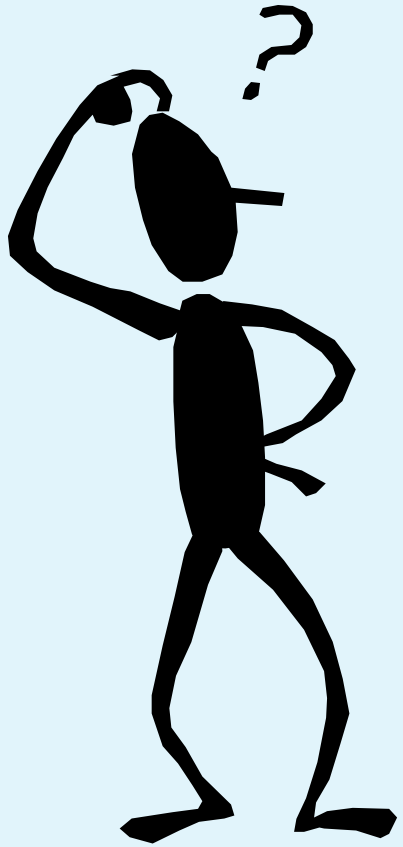


Medical Documentation

- CF2046 Field medical card;
- CF2046/A Field medical card (continuation);
- CF2047 Plastic protective envelope; and
- Field medical cards must be filled out on every casualty when the casualty is in CCT location/definitive care station.

Questions?





CASUALTY ASSOCIATED WITH HAZMAT SPILL

0830 TO 0955

HAZMAT OUTLINE

- Hazmat spill without casualties
- Hazmat spill with casualties
- Dress

HAZMAT SPILL WITHOUT CASUALTIES

- PA responds to the pipe closes up in Sickbay;
- CASUALTY CLEARERS close up outside Sickbay if HAZMAT Spill Clean-Up Team is piped;
- CDC will co-ordinate the dressing of the first two CC's to Level C and will dispatch them to the cold zone;
- A briefing with the On Scene Commander will occur before the CCT proceeds to the warm zone for decontamination of personnel leaving warm zone;

HAZMAT SPILL WITHOUT CASUALTIES

- CDC has second two CC dress half way in the event the first team needs to be relieved; (approx. 30 minutes)
- PA will brief CCT;
- PA may stand down CCT members except for the CDC and the two half dressed members; and
- Medical HAZMAT spill response team will only SECURE when the HAZMAT spill clean up team has been secured.

HAZMAT SPILL WITH CASUALTIES

- PA responds to the pipe as part of the RRT;
- CC close up outside Sickbay;
- CDC dispatches the first two CC members to the PA in the cold zone. They will bring first aid bag/C-collar, oxygen, and PA's bag to the PA. They can be used as runners for additional equipment or to escort walking wounded back to sickbay;

HAZMAT SPILL WITH CASUALTIES

- CDC will co-ordinate the dressing of the next two CC's to Level C and will dispatch them to the cold zone to be briefed by OSC and PA;
- CDC will have the next two CC dress half way in the event the first decontamination team needs to be relieved; (approx. every 30 minutes)

HAZMAT SPILL WITH CASUALTIES

- CDC and Medical Technician will make Sickbay ready to accept casualty;
- If there is a stretcher casualty, CCT I/C of evacuation will organize CCT personnel as follows: Designate 4 members with an Ashton-Water stretcher to muster in close proximity of the site and report to the PA as closed-up;

HAZMAT SPILL WITH CASUALTIES

- CCT I/C of the evacuation route will brief the remainder of the CCT members on the evacuation route and position members along water bulkhead doors/hatches. He will then report to PA as closed up to transfer casualty;
- A briefing with the On Scene Commander and PA will occur before the CCT proceeds to the warm zone for decontamination of personnel leaving and to decontaminate and treat casualties;

HAZMAT SPILL WITH CASUALTIES

- Once the casualty has been decontaminated and first aid rendered, CCT members will advise the PA;
- PA will monitor the transfer of the casualty from the warm zone to the cold zone, while ensuring that no cross contamination occurs;
- Casualty will proceed to sickbay as per evacuation route;
- CDC advises command of casualty's arrival to SB;

HAZMAT SPILL WITH CASUALTIES

- The 2 CCT members in the warm zone will remain on the dirty side and double bag all equipment used;
- CCT members will make themselves available to the on site coordinator; and
- They will proceed through decontamination procedures with the HAZMAT spill clean up team.

HAZMAT SPILL WITH CASUALTIES

- Its is imperative that the discoverer reports to Sickbay to be checked by the PA;
- Sickbay must receive an MSD sheet A.S.A.P.

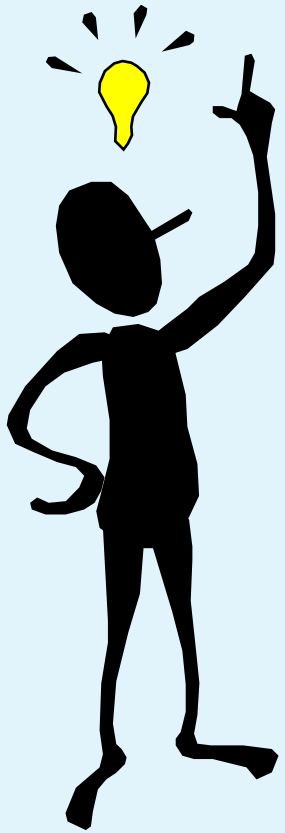
Ships PA to schedule towards weakness.

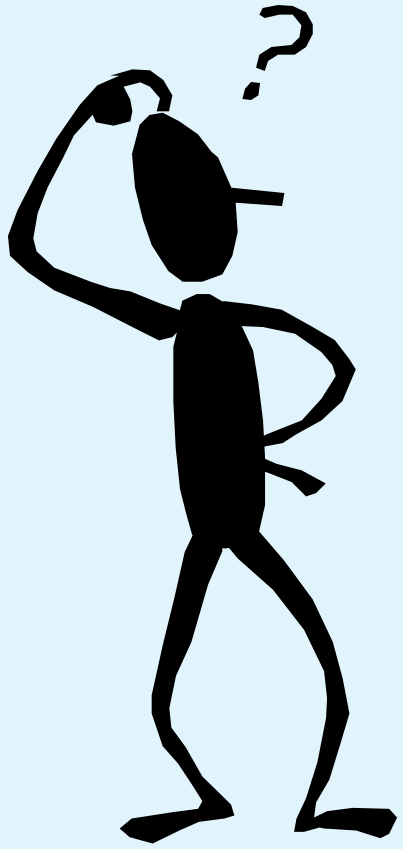
1015 TO 1130

ACT AS A MEMBER OF CCT

1230 TO 1330

Questions?





Practical – TBC once embarked

Sea Training